

Name	: Saroj Agarwal	Patient ID	: 0010663186
Date of Birth	: 27-08-1962	Age/Sex	: 63 Years/F
Performing Date	: 24-03-2026 06:42 PM	Order No.	: 0041235813
Referred By	: Shah Darshit	Report Date	: 25-03-2026 11:03 AM
Study Description	: CT Angiography - Pulmonary		

CT Angiography - Pulmonary

Technique: CT Pulmonary Angiography was performed following bolus tracking after intravenous administration of 40ml of contrast. Sagittal and coronal reformatted and MIP images were obtained.

Clinical details: Patient developed breathlessness and cough since 2-3 days. Developed hypoxia yesterday.

Findings:

Pulmonary arteries are dilated. The pulmonary trunk measures 35 mm. The right pulmonary artery measures 25 mm and left pulmonary artery measures 25 mm in calibre.

A saddle thrombus is seen extending from the main pulmonary trunk into the right and left main pulmonary artery branches.

On the left side, the filling defect is further seen extending into the upper and lower lobar segmental and subsegmental branches. Completely occluding thrombus is noted in proximal left upper lobar segmental branch;

Partially obstructive filling defects are seen in the right distal pulmonary artery, segmental and sub-segmental arterial branches of superior and all the basal segments [anterior, medial, posterior, lateral] of right lower lobe; Partially obstructive filling defects in all segmental and sub-segmental arterial branches of right middle and sub-segmental branches of right upper lobe.

Bilateral moderate pleural effusion is seen with partial subsegmental collapse of both lower lobes and lingular segment.

The visualised lung parenchyma reveals mosaic attenuation.

The right atrium and ventricle is mildly dilated. The interventricular septum is straight.

Visualized abdomen shows, two non-enhancing hypodense lesions noted in the right lobe of liver, largest ~3.2 x 2.8 cm with lobulated margins- benign in aetiology. Mild perisplenic fluid is noted. Tip of nasogastric tube is seen in duodenum.

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Remarks

Acute submassive pulmonary embolism with saddle thrombus involving the main pulmonary trunk into the right and left main pulmonary artery, lobar and segmental branches, associated with RV strain and dilated pulmonary arteries- suggesting secondary pulmonary hypertension.

Moderate bilateral pleural effusions with collapse of the underlying lung parenchyma.

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